

PROTOCOL UPDATES

July 1, 2025

REMOVED: BLS r-1 Airway Obstruction, info has been moved to R-01.

REMOVED: BLS r-2 Croup/Epiglottitis, info has been moved to R-01.

REMOVED: M-07 Subacute and Chronic Pain.

ADDED NEW: M-04 Pain Management.

ADDED NEW: P-14 Pediatric Pain Management.

ADDED NEW: P-15 Pediatric Airway Obstruction.

UPDATED: R-01 Airway Obstruction, verified correct version.

UPDATED: S-04 Needle Cricothyrotomy, verified correct version.

PROTOCOL AND POLICY UPDATES

October 15, 2025

UPDATED: Policy 5510 Ambulance Permit and Equipment Inspection Process, updated to newest version.

PROTOCOL AND POLICY UPDATES

January 5, 2025

UPDATED: Solano County Trauma Triage, replacing any previous trauma triage.

REMOVED: Attachment 1 – Solano County Trauma Triage Algorithm from policy 6105.
Solano County Prehospital Trauma Triage Plan.

MOVED: Solano County Trauma Triage is now located under the Trauma protocols.

PROTOCOL AND POLICY UPDATES

February 3, 2026

UPDATED: T-02 Traumatic Cardiac Arrest, clear wording indicating when to perform bilateral pleural decompression added.

Solano County Health & Social Services Department

Mental Health Services
Public Health Services
Substance Abuse Services
Older & Disabled Adult Services



Eligibility Services
Employment Services
Children's Services
Administrative Services

Ann Edwards, Director

Aaron E. Bair, MD, MSc
EMS Agency Medical Director

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www.solanocounty.com

Ted Selby
Emergency Services Bureau Chief

August 19, 2013

Colleagues,

Please, find the attached revision of the C3 and C4 cardiac arrest protocols.

Notably, they contain a substantive change regarding patients who have experienced out of hospital cardiac arrest (OHCA) and have had return of spontaneous circulation (ROSC).

Many of these patients (30-50%) have high-grade coronary lesions amenable to intervention but still may have equivocal (or unobtainable) ECGs en route to the hospital.

Per these protocol changes, these patients will now to be transported to the nearest STEMI receiving facility (SRC) regardless of ECG findings.

The rationale underlying this decision is as follows:

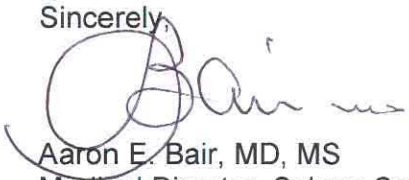
1. Immediate coronary angiography followed by successful coronary intervention has been shown to be an independent predictor for survival and improved neurological outcomes for patients with OHCA, *irrespective of the presence or absence of STEMI on the presenting ECG*.
2. The standard of care is quickly evolving, as evidenced by local counties that have recently made similar destination policy changes. A partial list includes, but is not limited to: Alameda, Contra Costa, Los Angeles, Napa, Orange, and Santa Clara.
3. This is unlikely to be a large additional burden on any one SRC as we estimate there is less than 1 patient per week with ROSC after OHCA in Solano County. This policy relates to a change in destination only for those patients who do not manifest STEMI changes on ECG. Hence, the implementation of this policy will only impact a subset of this patient population.

4. Given the county geography and location of current SRCs, we do not anticipate that this change in policy will add substantially to transport times (i.e. beyond a critical threshold of 30 min.).

In summary, these changes to policy are intended to improve care for the citizens of Solano County by lowering the barriers to early PCI in OHCA patients.

We anticipate carefully tracking the impact of this policy and very much appreciate your support and diligence with implementation.

Sincerely,

A handwritten signature in black ink, appearing to read 'A. Bair', with a large circular flourish at the beginning.

Aaron E. Bair, MD, MS
Medical Director, Solano County EMS

Solano County Health & Social Services Department

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Gerald Huber, Director

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Ted Selby
EMS Agency Administrator

Date: February 16, 2016
To: Advanced Life Support (ALS) Providers
From: Aaron Bair, MD, MS, EMS Agency Medical Director
Subject: Epinephrine 1:10,000 Concentration

The Solano County Emergency Medical Services (EMS) Agency is aware of the national and local shortage of Epinephrine 1:10,000 concentration. The EMS Agency and Physician's Forum has developed a short term solution to this issue: dilution of Epinephrine 1:1000 concentration. Once the Epinephrine is diluted it can be used in the same protocols as Epinephrine 1:10,000.

In order to dilute Epinephrine 1:1000 it must be preservative free. In general the ampules are preservative free. Multi-dose vials of Epinephrine are not preservative free and should not be used for the dilution procedure.

The procedure for dilution of Epinephrine 1:1000 is described below.

1. Ensure the Epinephrine 1:1000 is preservative free;
2. Draw 1ml of Epinephrine 1:1000 into a 10 ml syringe;
3. Using the same 10 ml syringe, draw 9 ml of preservative free normal saline and agitate the solution;
4. The solution in the 10 ml syringe contains 1 mg of Epinephrine 1:10,000.

Sincerely,


Aaron Bair, MD, MS
EMS Agency Medical Director